WPMH ISE 1 — Quick Revision Notes

Chapter 1: Introduction to Mental Health

Mental Health — Definition

> **WHO:** A state of well-being in which an individual realizes their abilities, copes with normal stresses, works productively, and contributes to community.

Continuum: Thriving → Healthy → Struggling → In Crisis (everyone moves along it)

Why It Matters at Work

Factor	Poor MH Impact	Good MH Impact
Productivity	Reduced output, errors	Higher efficiency
Absenteeism	Frequent sick leaves	Consistent attendance
Presenteeism	Present but disengaged (costs MORE than absenteeism)	Engaged & focused
Turnover	High attrition, costly	Better retention
Safety	More accidents, poor judgment	Better decisions
Financial	WHO: $1 trillion/yr lost globally	$1 invested → $4 returned

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Three Dimensions of Well-Being

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graph TD
    WB["Well-Being"] --> EWB["Emotional"]
    WB --> SWB["Social"]
    WB --> PWB["Psychological"]
    style WB fill:#264653,color:#fff
    style EWB fill:#2a9d8f,color:#fff
    style SWB fill:#e9c46a,color:#000
    style PWB fill:#e76f51,color:#fff
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Dimension	Key Aspects
Emotional	Managing emotions, coping with stress, optimism, self-awareness
Social	Relationships, belonging, collaboration, trust, supporting others
Psychological	Sense of purpose, personal growth, autonomy, self-acceptance

> **Signs of Social Well-Being:** Participating in team activities, positive relationships, willingness to collaborate, feeling included/valued.

Job Satisfaction ↔ Mental Health

Bidirectional: Good MH → higher satisfaction → better MH. Poor MH → low satisfaction → worse MH. They reinforce each other.

Resilience

Definition: Ability to adapt, recover, and grow from adversity/setbacks.

Components: Adaptability, Emotional regulation, Problem-solving, Social support, Positive mindset

Building it: Growth mindset, social connections, self-care, realistic goals, professional help when needed

> **Resilience strategy → Result:** Better stress tolerance, lower burnout, improved adaptability, stronger teams, setbacks viewed as learning.

Common Mental Health Disorders

Disorder	Key Symptoms	Workplace Impact
Depression	Persistent sadness, fatigue, loss of interest, worthlessness	Reduced productivity, withdrawal, absenteeism
Anxiety	Excessive worry, panic attacks, avoidance, restlessness	Avoiding meetings, perfectionism, missed deadlines
Burnout	Exhaustion, cynicism, reduced efficacy	Disengagement, high turnover
PTSD	Flashbacks, hypervigilance, emotional numbness	Difficulty focusing, avoidance, outbursts
OCD	Intrusive thoughts, repetitive rituals, excessive checking	Time consumed by rituals → team productivity loss
Bipolar	Mania (high energy) ↔ Depression cycles	Inconsistent performance, risky decisions

Physical Symptoms of Anxiety

Rapid heartbeat, chest tightness, shortness of breath, nausea, muscle tension, headaches, dizziness, sweating, insomnia, trembling

Myths vs Facts

Myth	Fact
"Just stress, not real"	Clinically diagnosed, biological basis
"They can snap out of it"	Not a choice; needs support & treatment

Myth	Fact
"Only weak people"	Affects everyone regardless of strength
"Depression = sadness"	Includes fatigue, cognitive impairment, sleep changes
"If they can work, they're fine"	Presenteeism is real and costly

Depression Misdiagnosed As:

Laziness, poor performance, personality trait, personal weakness → prevents help-seeking

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Recognizing Signs at Work

Category	Signs
Emotional	Sadness, mood swings, irritability, hopelessness
Behavioral	Absenteeism, declining quality, substance use, overworking
Physical	Fatigue, headaches, sleep issues, weight changes
Cognitive	Poor concentration, forgetfulness, negative self-talk
Social	Withdrawal, avoiding meetings, conflict with coworkers

Red Flags: Self-harm talk, feeling trapped, giving away possessions, extreme mood changes, reckless behavior

Social Withdrawal

- Eating alone, skipping events, minimal communication
- Worsens MH through isolation → need gentle check-ins, inclusive activities

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Legal & Ethical Considerations

Legal	Key Point
Anti-Discrimination	Can't discriminate based on MH status (ADA, RPWD Act 2016)
Reasonable Accommodations	Flexible hours, modified duties, quiet spaces, therapy time
Confidentiality	MH info is confidential; sharing without consent = legal action
OHS	Duty to maintain psychologically safe workplace
Workers' Compensation	Work-caused MH conditions may qualify
Right to Disconnect	Emerging laws on after-hours communication

Ethical	Key Point
Non-Stigmatization	Normalize help-seeking
Privacy	Never pressure disclosure

Ethical	Key Point
Duty of Care	Obligation to support distressed employees
Fair Treatment	MH status cannot influence career decisions

Duty of Care Breach Example

> Manager sees employee showing severe stress signs → ignores and increases workload → employee has breakdown → **breach** (failed to act + worsened situation). Consequences: legal liability, compensation claims, trust damage.

Confidentiality Breach Consequences

Lawsuits, loss of trust, stigma/discrimination, worsened MH, damaged org reputation

Psychological Safety

Feeling safe to take risks, voice opinions, admit mistakes without fear of punishment → encourages help-seeking, innovation, engagement.

Chapter 2: Creating a Supportive Work Environment

Promoting MH — Three Levels

Organizational: Policies, EAPs, leadership commitment, resource allocation **Team:** Supportive management, team building, open communication **Individual:** Self-care, skill development, help-seeking

Key Initiatives

EAPs, Mental Health Days, Wellness Programs, Flexible Work, Workload Management, Training, Peer Support, Safe Reporting

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Mental Health Literacy

Knowledge of MH conditions that aids recognition, management, prevention. Includes recognizing symptoms, knowing how to seek help, understanding risk factors, stigma-reducing attitudes.

Motivation & MH

Bidirectional: Good MH → motivation → engagement. Apply via: SMART goals, recognition, growth opportunities, autonomy, purpose connection.

Emotional Intelligence (EI)

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graph TD
    EI["Emotional Intelligence"] --> SA["Self-Awareness"]
    EI --> SR["Self-Regulation"]
    EI --> EM["Empathy"]
    EI --> SS["Social Skills"]
    EI --> MO["Motivation"]
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Self-Awareness → know your triggers | Self-Regulation → don't react impulsively | Empathy → notice struggling colleagues | Social Skills → communicate sensitively | Motivation → maintain drive

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Stigma — 3 Types

Type	Manifestation
Public	Gossiping, labeling as "weak"
Self	Hiding condition, shame, not seeking help
Institutional	No MH policies, insurance gaps, penalizing MH leave

Reducing Stigma

Leadership example → Education → Appropriate language → Normalize conversations → Celebrate help-seeking → Anti-bullying policies → Mental Health Champions

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Zero-Tolerance Bullying — HR Steps

1. Define bullying clearly
2. Multiple reporting channels (anonymous)
3. Investigate promptly & fairly
4. Enforce consequences (no seniority exceptions)
5. Protect complainant (anti-retaliation)
6. Train all employees + managers
7. Monitor via surveys & exit interviews
8. Support victims (EAP, schedule adjustments)
9. Leadership endorsement

Diversity, Inclusion & MH

Inclusive policies → culturally sensitive counseling, multilingual resources. Exclusion/discrimination = major stressors. Belonging protects against depression/anxiety.

Key Policies

Policy	Core Elements

Policy	Core Elements
MH Policy Statement	Commitment, scope, responsibilities
Stress Management	Risk assessment, interventions, monitoring
Anti-Bullying	Definitions, reporting, investigation, consequences
Accommodations	Request process, types, confidentiality
Return-to-Work	Phased return, modified workload, no penalty
Confidentiality	Data storage, access limits, consent
Crisis Response	Emergency contacts, trained responders, post-incident support

Resources

EAPs (6-8 sessions free), In-house counselors, External referrals, Helplines (iCall, Vandrevala Foundation, 988), Self-help apps (Headspace, Calm), Support groups, Financial wellness programs

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Chapter 3: Procrastination, Stress & Work-Life Balance

Procrastination

> NOT laziness — it's an **emotional regulation problem** (avoiding negative emotions tied to a task).

6 Types

Type	Pattern
Perfectionist	Won't start until it can be "perfect"
Dreamer	Big ideas, avoids execution
Avoider	Fear of failure/success
Crisis-Maker	"I work best under pressure"
Busy Procrastinator	Does easy tasks, avoids hard ones
Indecisive	Paralysis by analysis

Strategies

Break tasks down, 2-Minute Rule, Pomodoro (25/5), Set artificial deadlines, Eat the Frog (hardest first), Remove distractions, Self-compassion, Reward system, Accountability partner

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Stress

Eustress (positive, motivating) vs **Distress** (negative, harmful)

> **Yerkes-Dodson Law:** Performance ↑ with stress to an optimal point, then ↓ sharply.

Common Stressors

Excessive workload, Job insecurity, Poor relationships, Lack of control, Role ambiguity, Organizational change, Poor environment, Work-life imbalance, Lack of recognition, Discrimination

GAS (Hans Selye) — 3 Stages

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graph LR
    A["Alarm
(Fight/Flight)"] --> B["Resistance
(Adapting)"]
    B --> C["Exhaustion
(Breakdown)"]
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    style B fill:#e9c46a,color:#000
    style C fill:#e76f51,color:#fff
```

Effects of Stress

Physical	Behavioral	Emotional	Cognitive
Headaches, fatigue, insomnia, high BP, weak immunity	Absenteeism, substance use, withdrawal, aggression	Irritability, anxiety, depression, helplessness	Poor concentration, forgetfulness, negative thinking

Coping Mechanisms

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graph TD
    CM["Coping"] --> PF["Problem-Focused"]
    CM --> EF["Emotion-Focused"]
    CM --> AF["Avoidance-Focused"]
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    style PF fill:#2a9d8f,color:#fff
    style EF fill:#e9c46a,color:#000
    style AF fill:#e76f51,color:#fff
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Type	Examples	When Effective
Problem-Focused	Planning, seeking help, delegating, negotiating	Situation IS within your control ☑
Emotion-Focused	Meditation, journaling, reframing, exercise	Situation NOT in your control ☑
Avoidance-Focused	Denial, substance use, distraction	❌ Temporary relief, worsens long-term

Positive coping: Exercise, mindfulness, social support, journaling, therapy **Negative coping:** Substance abuse, isolation, aggression, denial

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Stress Management Techniques

Category	Techniques
Physical	Exercise, sleep (7-9 hrs), nutrition, deep breathing
Psychological	Mindfulness, CBT reframing, journaling, professional help
Time Management	Eisenhower Matrix, say "No", delegate, SMART goals
Social	Talk to someone, hobbies, digital detox

Eisenhower Matrix

- **Q1 (Urgent+Important):** DO IT — crises, deadlines
- **Q2 (Not Urgent+Important):** SCHEDULE — planning, self-care, learning ← *spend more time here*
- **Q3 (Urgent+Not Important):** DELEGATE — interruptions, some emails
- **Q4 (Not Urgent+Not Important):** ELIMINATE — time wasters

Time Management for Stress Reduction

Prioritize (Eisenhower/ABCDE), SMART goals, plan daily/weekly, delegate, say no, avoid multitasking, schedule buffer time

Organizational Interventions

Primary (Prevention): Job redesign, reduce workload, increase autonomy **Secondary (Coping):** Training, workshops, mindfulness, fitness **Tertiary (Treatment):** EAPs, counseling, return-to-work support

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Work-Life Balance vs Integration

Balance	Integration
Clear boundaries	Fluid blending
"Leave work at work"	Handle both throughout day
Fixed schedules	Remote/hybrid friendly

Strategies

Flexible scheduling, clear log-off times, generous PTO, manager training, results-oriented culture, family-friendly policies, workload audits, encourage PTO use

Signs of Poor Balance

Working late/weekends constantly, no PTO use, declining social events, chronic fatigue, guilt when not working, declining quality despite more hours

> **Manager's Role:** Check in proactively, model healthy behavior, redistribute work before burnout.